

PRODUCT LITERATURE

METACIN TABLET 500 MG

Each tablet contains

Paracetamol 500 mg

Preservative

Sodium Benzoate 0.1%w/w

Description

White coloured odourless, round tablet with ZONTRON 500 and a score embossed on one side.

Pharmacodynamics Properties

Mechanism of action

Paracetamol is an analgesic antipyretic and mild anti-inflammatory. The mechanism of action is probably similar to that of aspirin and dependent on the inhibition of prostaglandin synthesis. This inhibition appears, however, to be on a selective basis.

Pharmacokinetics Properties

Absorption:

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract. The concentration in plasma reaches a peak in 30 to 60 minutes and the plasma half-life is 1 – 4 hours after therapeutic doses. Paracetamol is metabolised in the liver and excreted in the urine, mainly as the glucuronide and sulphate conjugates. A hydroxylated metabolite may accumulate and cause liver damage. Considerable “first pass” inactivation takes place in the gut wall and liver.

Distribution

Paracetamol is relatively uniformly distributed throughout most body fluids. Binding of the drug to plasma proteins is variable; 20 to 30% may be bound at the concentrations encountered during acute intoxication.

Elimination

Following therapeutic doses 90 - 100% of the drug may be recovered in the urine within the first day. However, practically no paracetamol is excreted unchanged and the bulk is excreted after hepatic conjugation.

Indication

Analgesic and antipyretic. Indicated for the relief of mild to moderate pain and to reduce fever.

Route of administration :

Oral.

Recommended Dose

Adult: 500 – 1000 mg every 4-6 hours, maximum of 4 g daily.

Children: 6-12 years: 250-500 mg per dose.

Repeat every 4-6 hours when necessary.

Maximum of 4 doses in 24 hours

Contraindications

Hypersensitivity to paracetamol or any of the other excipients.

Warning and Precautions

**This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.**

• **Allergy alert:** Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away

- Care is advised in the administration of paracetamol to patients with impaired renal or hepatic function. The hazard of overdose is greater in those with non-cirrhotic alcoholic liver disease.
- Do not exceed recommended dose.
- Patients should be advised not to take other paracetamol-containing products concurrently.
- Patients should be advised to consult a doctor if they suffer from non-serious arthritis and need to take painkillers every day.
- Patients should be advised to consult their doctor if their headaches become persistent.
- If symptoms persist the patient should consult his/her doctor.
- Keep out of the sight and reach of children.

Interactions with Other Medicaments

Hepatotoxicity is enhanced by alcohol, phenobarbitone and doxorubicin. Paracetamol increases blood concentrations of aspirin.

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by cholestyramine.

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

Pregnancy and Lactation

Pregnancy:

A large amount of data on pregnant women indicate neither malformative, nor feto/neonatal toxicity. Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy however it should be used at the lowest effective dose for the shortest possible time and at the lowest possible frequency.

Breast feeding:

Paracetamol is excreted in breast milk but not in a clinically significant amount. Available published data do not contraindicate breast feeding.

Side Effect

Adverse events of paracetamol from historical clinical trial data are both infrequent and from small patient exposure. Accordingly, events reported from extensive post- marketing experience at therapeutic/labelled dose and considered attributable are tabulated below by system class. Due to limited clinical trial data, the frequency of these adverse events is not known (cannot be estimated from available data), but post- marketing experience indicates that adverse reactions to paracetamol are rare and serious reactions are very rare.

Post marketing data:

Body System	Undesirable effect
Blood and lymphatic system disorders	Thrombocytopenia Agranulocytosis
Immune system disorders	Anaphylaxis Cutaneous hypersensitivity reactions including skin rashes, angioedema and Stevens Johnson syndrome/toxic epidermal necrolysis
Respiratory, thoracic and mediastinal disorders	Bronchospasm*
Hepatobiliary disorders	Hepatic dysfunction

*There have been cases of bronchospasm with paracetamol, but these are more likely in asthmatics sensitive to aspirin or other NSAIDs.

Symptoms and Treatment of Overdose

Liver damage is possible in adults who have taken 10g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Risk Factors

If the patient

a. is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

b. regularly consumes ethanol in excess of recommended amounts.

Or

c. is likely to be glutathione depleted e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms

Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute

tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management

Immediate treatment is essential in the management of Paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of the overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N- acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24 hours from ingestion should be discussed with the liver unit.

Packing

Packed in blister-packs of 10 x 10's / Box

Storage Conditions

Store below 30°C in a dry place, protected from direct light.
Keep medicine out of reach of children

Expiry: 3 years from date of manufacture

Name and Address of Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Product Registration Holder/ Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Malaysian Drug Registration No. :

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